PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Study protocol of a systematic review and qualitative evidence synthesis using two different approaches: Healthcare related needs and desires of older people with post-stroke aphasia
AUTHORS	Pohontsch, Nadine; Meyer, Thorsten; Eisenmann, Yvone; Metzendorf, Maria-Inti; Leve, Verena; Lentsch, Veronika

VERSION 1 – REVIEW

REVIEWER	Jane Noyes Bangor University and Cochrane UK
REVIEW RETURNED	01-May-2020

GENERAL COMMENTS	Thank you for asking me to review this interesting methodological protocol, which although topical raises a lot of fundamental methodological issues. It is really good to see a team wanting to compare methods. The aim (although not explicitly stated) is to conduct a methodological exemplar comparing two methods of qualitative evidence synthesis that have similarities and differences and sit in different epistemological locations on the idealist to realist spectrum. Two methods are selected meta-ethnography and Thomas and Harden's thematic synthesis. The protocol raises a number of methodological concerns that require resolution: 1. If conducting the work as a methodological exemplar, this this should be an explicit aim and there should be an additional methodological question to address. Please address. 1.1 The title needs looking at again. This is not a double meta-synthesis. It is a methodological exemplar comparing the application of two qualitative evidence synthesis methods. Please address. 1.2 Of particular concern, the framework to compare and contrast the respective findings from the syntheses will be described in detail elsewhere. This means that the current protocol is incomplete and the main purpose cannot be achieved. Please address by including the methods for developing the structured framework to compare the two methods. Otherwise this protocol is largely redundant in purpose. I acknowledge that this framework is likely to evolve during the conduct of the reviews, but the protocol at least should outline the author's theoretical conceptualisation and ideas on this and articulate the processes looking forward. This is the fundamental methodological component of this protocol. Otherwise its just a protocol for the conduct of the same review using two reasonably similar methods. 1.2.1 Thus far methodological exemplars have compared methods that start at different positions and process the data in completely different ways to produce different types of synthetic product, such as framework or best fit framework synthesis compared with thematic synthesis. The manuscript could be further

	strengthened by further articulating the epistemological differences between the two selected methods and how the team plan to 'operationalise' their thinking and processing of these epistemological differences in processing the data in the respective reviews. This issue is particularly important as both selected methods have more similarities than differences in their anticipated purpose and data processing – that is both methods are designed to produce new theory and both methods share similar aspects of data processing and synthesis so the main key difference is the epistemological location. 2. The authors appear to misinterpret the current guidance on choosing a qualitative evidence synthesis methodology. 'Albeit theory-based recommendations for deciding which type of synthesis to choose for certain qualitative research questions exist [14–16], to date there is no empirical evidence showing which approach serves best in qualitative health care research.' This sentence demonstrates a lack of understanding of the published guidance in that the choice of qualitative evidence synthesis method is dependent on a number of methodological considerations (such as the question, number and type of studies, richness and quality of the data etc). The RETREAT framework has been specifically designed to guide authors to select the most appropriate qualitative evidence synthesis method for their specific healthcare context. Please reread: https://www.ncbi.nlm.nih.gov/pubmed/29548841 and https://www.integrate-hta.eu/wp-content/uploads/2016/02/Guidance-on-choosing-qualitative-evidence-synthesis-methods-for-use-in-HTA-of-complex-interventions.pdf 2.1 The key point here is that although the methods for the search strategy would be the same irrespective of the method of synthesis, you may select a different qualitative evidence synthesis method depending on the type and richness of available studies and you may sample the studies differently depending on the qualitative evidence synthesis method selected. A meta-ethnography question tends to be constructed differently than a thematic synthesis question (for example). In the current protocol, all the methods of question formulation, data identification, selection and processing are generic up until the point of synthesis and this would not happen if the syntheses were conducted as different and separate products. It needs acknowledging that specific processes would not happen in this way if the two syntheses were conducted separately. This may also lead to a distortion of the methodological exemplar and its findings. By using PRISMA P which is designed for quant SRs and meta-analyses- the authors have been taken down the path of writing a protocol as if all the methodological uncertainties were known. For example, current Cochrane guidance suggests that final selection of a method of qualitative evidence synthesis may not be appropriate until the pool of available evidence is known and an assessment of its methodological quality and conceptual richness can be made. The authors may want to look at the Cochrane template for undertaking a qualitative evidence synthesis (see link below). Not all aspects will be relevant (such as integrating the QES with a Cochrane intervention effect review, but other issues not covered in the current protocol such as author reflexivity should be included in the current protocol. See: https://epoc.cochrane.org/sites/epoc.cochrane.org/files/public/uploads/Resources-for-authors2017/epoc_qes_protocol_and_review_template.pdf The current protocol will only therefore 'work' if it is identified as a methodological exemplar and it is acknowledged that if conducted separately then the reviews would likely look different. For example, in the current protocol the assumption is that the review team want to use the same evidence (ie studies) to conduct both types of synthesis. In reality, if conducted separately different types of evidence may be selected for the respective qualitative evidence synthesis method. Please address. 2.2 Linked to 2.1 The question is exceptionally broad "What are the needs and desires related to health care of people suffering from aphasia after stroke?" This

	is not a great QES question as it is so broad and could cover just about anything for any group of people with stroke and aphasia in any context or health system. A more specific and meatier question could enable a much richer and more meaningful methodological exemplar. Plus, even though it is very broad, as specified, the question is more suited to a thematic synthesis whereby you might include conceptually thin clinical study reports of relatively small scale qualitative studies. For a meta-ethnography you need at least some conceptually rich qualitative studies in order to develop new theory (the main purpose of conducting a meta-ethnography). Although both meta-ethnography and Thomas and Harden's thematic synthesis method have the capacity to develop new theory or analytical level interpretations not seen in primary studies, it is not clear what added value a new theory of stroke survivor's needs and desires from healthcare would add to what is already known. In summary, the question as currently specified seems the least likely to offer the most opportunities to draw out the differences in the findings from the two methods, when the differences are likely to be at theoretical level. Please address. 2.2.1 The 'I' in PICO relates to the 'phenomenon of Interest'. Please make this clear in the protocol. 3. Please refer to 'finding's and not 'results'. This is a qualitative exemplar so please use the language most relevant to the qualitative paradigm. 4. The lack of critical appraisal is problematic for the reasons already outlined. The review authors will need to make an assessment of methodological limitations in included studies (such as thick thin data and conceptually rich poor data) in order to understand and compare the methodological processing of data extracted from included studies. If sampling, the author team may also want to sample based on an assessment of methodological limitations in primary studies. Whilst I accept that there is ongoing discussion about the appropriateness of current appraisal tools, and whilst we await the release of the new CAMELOT tool, Cochrane has however provided guidance on a narrow set of quality appraisal tools that are appropriate for use in qualitative evidence syntheses if they are used to identify methodological limitations. 5. Application of GRADE CERQual may provide another opportunity for the authors to further articulate the differences in the confidence in review findings from the respective application of QES methods. 6. The author team have not cited the eMERGe guidance on reporting meta-ethnography or the latest methodological articles on articulating and operationalising the complex analytical phases of meta-ethnography. 7. Of particular note, both the Thomas and Harden method of thematic synthesis and meta-ethnography have commonly been poorly applied and not conducted as intended. It is common to see published reviews that 'use' these methods but various aspects have been adapted, skipped over or misunderstood. To produce a convincing methodological exemplar, the authors will need to stay faithful to the methods as originally intended and follow the guidance of the originators or authors whose methodological work to further articulate the methods is considered high quality by the originators. Thank you once again for asking me to review this interesting protocol. Overall, comprehensive revision and additional thinking is required.

REVIEWER	Assoc Prof Mandy Stanley Edith Cowan University, Australia
REVIEW RETURNED	07-May-2020

GENERAL COMMENTS	Thank you for the opportunity to review the manuscript. I am intrigued by the ideas of analysing the data using two different approaches to synthesis and look forward to reading the outcome. It is indeed an interesting question. However by naming it a "double meta-synthesis" seems to me to be misleading. Double synthesis implies to me that you are analysing the data twice over, and whilst essentially you are, it is more that you are approaching

	the same data in two different ways. It may be a matter of semantics but I think it is important to reflect on. You say you will search at least four databases. Will it be four or more than four? There appears to be some material out of place in the ethics and dissemination section of the abstract. Will you inform policy? How? Much more likely to inform best practice. I was interested in the decision to focus on older people (rather than elderly) yet you cite statistics about the frequency of stroke from age 40-79. The manuscript needs review for a number of missing words and some smoothing out of English language. I got to the end of the introduction section and was unclear how the findings about needs related to people with aphasia, and why the review needed to be conducted. This needs to be much more explicit, not just the methodological curiosity in trying two different analysis approaches. In the methods section there is a statement about ethics which is not necessary here as stated later. Databases and search terms: state the languages to be searched in. Table 2 objective Is it consented or consensus? Data extraction - will you also extract study design? You talk about the distinction between first and second order constructs not being clear, what approach will you use? The section on how you chose the two different approaches needs to come earlier to make a bit more sense. Am I correct in understanding that the analysis will be done by two independent teams? I was wondering how you would prevent the findings from one influencing the other especially if they are both discussed with the research network. The discussion is a bit weak. Aphasic patients should be people living with aphasia. You suggest it will inform more patient-centred ways but how do you know current ways are not? The findings should provide a more in-depth conceptual understanding of what is important for people living with aphasia. I am also intrigued about how you will manage reporting. If you publish one approach will that make publication of the other approach more difficult? It clearly will lead to an interesting methodological paper however that shouldn't be the primary goal.

REVIEWER	Betje Schwarz German Pension Insurance Berlin-Brandenburg, Germany
REVIEW RETURNED	13-May-2020

GENERAL COMMENTS	A very good study protocol of a very interesting and promising study! I have only one request for clarification: in the section 'Authors Contributions' you state, that "All authors contributed to […] the risk of bias assessment." Which bias assessment? You also state, that you will not assess the methodological quality of the primary studies: "Even though Thomas & Harden [28] describe quality appraisal as a step in their approach to thematic synthesis it has no consequences for study inclusion or exclusion or the weighing of results. Following Noblit & Hare [29] meta-ethnography does not necessarily include quality appraisal of the primary studies. Therefore, we decided not to include explicit quality assessment of the included studies."

VERSION 1 – AUTHOR RESPONSE

Reviewer: 1

Reviewer Name

Jane Noyes

Institution and Country

Bangor University and Cochrane

UK

Reviewer(s)' Comments to Author:

Please state any competing interests or state ‘None declared’: None declared

Commentary Reviewer 1	Answer	Manuscript Page
1. If conducting the work as a method- ological exemplar, this this should be an explicit aim and there should be an additional methodological question to address. Please address	Dear Mrs. Noyes, thank you very much for your time and efforts spent on reviewing our manuscript. Your comments were very helpful to us. The primary aim of the manuscript is to function as a study protocol for two meta- syntheses which are aimed at answering a specific research question. The secondary aim of our project is to compare the process and results of the two meta-synthesis methods. This topic will be addressed in further publications. We appreciate your interest in the method of comparison, but the development of the methodology is part of our future study. Right now we would prefer our manuscript to be viewed as a study protocol for two seperate meta-syntheses. Either way, we will give an out- look of the route we will travel in comparing both methods. Further refinements of	5

	this approach will be described in later methodological publications. To clarify this, we reformulated the research question, please see under “Objectives” in the introduction.	
1.1 The title needs looking at again. This is not a double meta-synthesis. It is a methodological exemplar compar- ing the application of two qualitative evidence synthesis methods. Please address.	As already mentioned, our aim is twofold: Gathering the evidence of qualitative stud- ies on a certain topic (which is described here in detail) AND *later on* examining the differences (and similarities) of the different methods. The term “double-metasyn- thesis” may be misleading, therefore we changed the title. New Title: Healthcare related needs and desires of people with post-stroke aphasia: study pro- tocol of a systematic review and qualitative evidence synthesis using two different approaches	1
1.2 Of particular concern, the frame- work to compare and contrast the re- spective findings from the syntheses will be described in detail elsewhere. This means that the current protocol is incomplete and the main purpose can- not be achieved. Please address by in- cluding the methods for developing the structured framework to compare the two methods.	Please see the new “Objectives” section and “Constant comparison process” in the methods section.	5,9

Otherwise this protocol is largely re- dundant in purpose.	Before starting our research, we conducted a search for existing qualitative synthesis in order to make sure, our research is NOT redundant. This revealed, that our syn- thesis was the only one focussing on needs and desires of older people with post- [redacted] stroke aphasia with respect to the health care system. We described the differences [redacted] between this three reviews and ours in the introduction.	3

Commentary Reviewer 1	Answer	Manuscript Page
I acknowledge that this framework is likely to evolve during the conduct of the reviews, but the protocol **at** **least should outline the author's theoretical conceptu-** [redacted] **alisation and ideas on this and articulate the processes** **looking forward**. This is the fundamental methodologi- [redacted] cal component of this protocol. Otherwise its just a pro- tocol for the conduct of the same review using two rea- sonably similar methods	Please see section "data extraction and analysis" and "Constant comparison process"	9
1.2.1 Thus far methodological exemplars have com- pared methods that start at different positions and pro- cess the data in completely different ways to produce	Please see "Methods for synthesizing qualitative research: which one to choose when?"	

different types of synthetic product, such as framework or best fit framework synthesis compared with thematic synthesis. **The manuscript could be further strength-** **ened by further articulating the epistemological differ-** **ences between the two selected methods and how the** **team plan to 'operationalise' their thinking and pro-** **cessing of these epistemological differences in pro-** **cessing the data in the respective reviews.** This issue is particularly important as both selected methods have more similarities than differences in their anticipated purpose and data processing – that is both methods are designed to produce new theory and both methods share similar aspects of data processing and synthesis so the main key difference is the epistemological location.		3
2. The authors appear to misinterpret the current guid- ance on choosing a qualitative evidence synthesis meth- odology. 'Albeit theory-based recommendations for de- ciding which type of synthesis to choose for certain qual- itative research questions exist [14–16], to date there is no empirical evidence showing which approach serves best in qualitative health care research.' This sentence	We referred to a lack of empirical (sic!) evidence in the comparison of different QES approaches. The valuable published RETREAT guid- ance is based on a review of experiences and deliberations of au- thors regarding different potentially relevant dimensions that should be acknowledged in the decision for or against certain QES approaches. It is not a substitute for an empirical comparison, which our projects aims at. We used the RETREAT framework to get	

demonstrates a lack of understanding of the published guidance in that the choice of qualitative evidence syn- thesis method is dependent on a number of methodo- logical considerations (such as the question, number and type of studies, richness and quality of the data etc). The RETREAT framework has been specifically designed to guide authors to select the most appropriate qualita- tive evidence synthesis method for their specific healthcare context. Please reread: https://www.ncbi.nlm.nih.gov/pubmed/29548841 and https://www.integrate-hta.eu/wp-content/uploads/2016/02/Guidance-on-choosing-qualitative-evidence-synthesis-methods-for-use-in-HTA-of-complex-interventions.pdf	an overview over existing QES-methods. We have tried to make this more explicit in the introduction under “Methods for synthesizing qualitative research: which one to choose when?”	3

		Page
2.1 The key point here is that although the methods for the search strategy would be the same irrespec- tive of the method of synthesis, you may select a different qualitative evidence synthesis method de- pending on the type and richness of available stud-	We are quite aware that a comparison of different QES methods can only be made with some methodological limitations. From a pure con- ceptual or even idealistic perspective, a strict comparison is hardly pos- sible or only possible for two quite similar approaches. We have decided to choose approaches similar enough to be able to apply to the same	

ies and you may sample the studies differently de- pending on the qualitative evidence synthesis method selected. A meta-ethnography question tends to be constructed differently than a thematic synthesis question (for example). In the current pro- tocol, all the methods of question formulation, data identification, selection and processing are generic up until the point of synthesis and this would not happen if the syntheses were conducted as differ- ent and separate products. It needs acknowledging that specific processes would not happen in this way if the two syntheses were conducted sepa- rately. This may also lead to a distortion of the methodological exemplar finding and its s.	research question (leaving it open to adopt the question during the course of the study if necessary), and to base (!) it upon the same set of studies, which does not predeterime the final selection of studies or the priorities given to each study. We assume, however, that the reality of choice over QES approaches is in practice much more dependent on cri- teria that go beyond methodological considerations – such as personal background and familiarity with a method, personal preferences of team members etc. In practice, therefore, boundaries between differ- ent QES approaches might be quite blurred in terms of concepts, meth- ods or terms. Since there is no “ideal” comparison in this vein, we find it appropriate to make a few restrictions to the respective QES method- ologies in order to allow for a substantive comparions. It is well acknowledged that these restrictions should be an explicit part of the approach to comparison.	11
By using PRISMA P which is designed for quant SRs and meta-analyses- the authors have been taken down the path of writing a protocol as if all the methodological uncertainties were known. For ex- ample, current Cochrane guidance suggests that fi- nal selection of a method of qualitative evidence	The EPOC-guidance offers very useful aspects to consider when writing an abstract or a plain language summary. The aspects the EPOC- guidance recommends for writing abstracts are fulfilled. For our paper is a protocol and does not belong to the systematic of Cochrane Reviews, we did not search for guidance in this area. Never- theless we decided to expand our protocol with a statement on author	10

synthesis may not be appropriate until the pool of available evidence is known and an assessment of its methodological quality and conceptual richness can be made. The authors may want to look at the **Cochrane template for undertaking a qualitative** **evidence synthesis** (see link below). Not all aspects will be relevant (such as integrating the QES with a Cochrane intervention effect review, but other is- sues not covered in the current protocol such as **au-** **thor reflexivity should be included in the current** **protocol**. See: https://epoc.cochrane.org/sites/epoc.cochrane.or g/files/public/uploads/Resources-for-au- thors2017/epoc_qes_protocol_and_review_template.pdf	reflexivity, which is suggested to be embedded in plain language sum- maries. Please see “Author background and reflexivity” in the method’s section. The final review will incorporate a reflexive statement to disclose how our positons may have influenced the review and whether our attitudes toward the research topic changed during the review. Please see “Au- thor background and reflexivity” in the method’s section.	10
The current protocol will only therefore 'work' if it is identified as a methodological exemplar and it is acknowledged that if conducted separately then the reviews would likely look different. For example, in the current protocol the assumption is that the re- view team want to use the same evidence (ie stud-	Please see new text under „Objectives“ and „Constant comparison pro- cess”.	5, 9

ies) to conduct both types of synthesis. In reality, if conducted separately different types of evidence may be selected for the respective qualitative evi- dence synthesis method. Please address.		

Commentary Reviewer 1	Answer	Manuscript Page
2.2 Linked to 2.1 **The question is exceptionally** **broad** “What are the needs and desires related to health care of people suffering from aphasia after stroke?” This is not a great QES question as it is so broad and could cover just about anything for any group of people with stroke and aphasia in any con- text or health system. A more specific and meatier question could enable a much richer and more meaningful methodological exemplar.	Broad questions in qualitative synthesis are not exceptional, because in the beginning it is unclear which – if any – qualitative evidence does exist. “The exploratory process means that initial qualitative review questions may be broad in order to map what is known, before formu- lating or refining questions” (Harris et al. 2018, p 4). Moreover, we do not see, why our question should be too broad, while questions labelled as “useful” in Booth et al. 2018, p. 48-49 seem to be quite similar to ours: Question 1. “To analyze, through an interpretative systematic review of qualitative studies, the meaning and motivation of the WTHD in pa- tients with chronic illness or advanced disease” Question 2: “What are the experiences of adolescents and young adults living with everyday pain?”	

	We could not discover the fundamental difference to our research question (“What are the needs and desires related to health care of people suffering from aphasia after stroke?”) To ensure that there are at least some qualitative studies in this area, we conducted searches in advance (for primary studies as well as for reviews), that resulted in some, but not exhaustive evidence. By study- ing similar – not identical – reviews found, we realized that the corpus of the existing evidence is not very large. The three qualitative synthesis focusing on questions related to ours included 51 (Pindus et al. 2018), 31 (Manning et al. 2019) and 3 studies (Hilari et al. 2012). Only Hilari et al. (2012) and Manning et al. (2019) restricted their research on stroke survivors with aphasia (but not on older people). Pindus et al. included subjects with and without aphasia. Therefore, to formulate a narrower question would probably result in very few hits in the database-search or after application of in- and exclusion criteria.	3
Plus, even though it is very broad, as specified, the question is more suited to a thematic synthesis whereby you might include conceptually thin clini- cal study reports of relatively small scale qualitative studies.	Conceptually “thin” studies are not the aim of our search, albeit they do not violate research findings. They just do not add much to the synthesis findings. Using the concepts of “needs and desires” opens up the possi- bility to use both a rather descriptive approach – such as the thematic analysis –and a more interpretative approach, such as the meta-ethnog- raphy approach. Therefore, we do not really comply with the point	

	raised here.	
For a meta-ethnography you need at least some conceptually rich qualitative studies in order to de- velop new theory (the main purpose of conducting a meta-ethnography). Although both meta-ethnog- raphy and Thomas and Harden's thematic synthesis method have the capacity to develop new theory or analytical level interpretations not seen in primary studies**, it is not clear what added value a new the-** **ory of stroke survivor's needs and desires from** **healthcare would add to what is already known. In** **summary, the question as currently specified** **seems the least likely to offer the most opportuni-** **ties to draw out the differences in the findings** **from the two methods, when the** differences are likely to be at theoretical level. Please address	The point of departure for our research question comes from the prob- lems we see in the German health care system with regard to cross- sectoral care. While the different health care institutions caring for stroke patients all have their merits, there is a huge problem in coordi- nation of care and taking responsility of care, which is often left to pa- tients overwhelmed with their situation. Especially patients with apha- sia have a literal problem to raise their voice. Therefore, we want to learn from qualitative studies about the needs and desires (i.e. objec- tive needs as reported by the patients themselves, as well as subjective needs related to goals, preferences, expectations etc.) in order to in- form present health care approaches that aim to integrate the different care stakeholders, especially rehabilitation, nursing and primary helath care services. From our view, we find it difficult to predict the added value(s) of our analyses. At least, our review will provide a systematic overview on “what is known”, and depending on the original studys and the type of analysis also what can be added to the knowledge of the primary stud- ies.	
2.2.1 The ‘I’ in PICO relates to the ‘phenomenon of Interest’. Please make this clear in the protocol	„phenomenon of“ added, see PICoS	6

Commentary Reviewer 1	Answer	Manuscript Page
3. Please refer to 'finding's and not 're- sults'. This is a qualitative exemplar so please use the language most relevant to the qualitative paradigm.	As our synthesis will lead to different types of results (related to the question of needs and desires of people with post-stroke aphasia (1) and related to methodology (2)) the use of the term "finding" in the sense Lewin et al. (2015) defined it:*"For the* *purposes of CERQual, we define a review finding as an analytic output from a quali-* *tative evidence synthesis that, based on data from primary studies, describes a phe-* *nomenon or an aspect of a phenomenon.*" Refers only to the first objective. But we acknowledge that in the paragraph „Constant comparison process", the term "find- ing" is more suitable and revised the text.	9
4. The lack of critical appraisal is prob- lematic for the reasons already out- lined. The review authors will need to make an assessment of methodologi- cal limitations in included studies (such as thick thin data and conceptually rich poor data) in order to understand and compare the methodological pro- cessing of data extracted from in-	Rose Barbours' paper from 2001 in the BMJ pinpointed the problem of using quality appraisal tools for so-called qualitative studies. As we have analysed in a paper of the qualitative methods working group within the German network on Health Ser- vices Research (Stamer et al. 2015), there is a substantial problem in a general ap- praisal of methodologically different studies, although they firm under the heading of being "qualitative". To our knowledge the classification of data as "thin" or "thick" in qualitative synthe- sis is not necessarily a result of good methodological quality but of the insightfulness of the data (Toye et al. 2013). Pindus et al. (2018), who did quality appraisal, found	

cluded studies. If sampling, the author team may also want to sample based on an assessment of methodological limitations in primary studies. Whilst I accept that there is ongoing discussion about the appropriateness of current appraisal tools, and whilst we await the release of the new CAMELOT tool, Cochrane has however provided guidance on a narrow set of quality appraisal tools that are appropriate for use in qualitative evidence syntheses if they are used to identify methodological limitations.	that "while the included studies achieved good quality scores, many failed to provide sufficient contextual detail". Despite our reservations against critical appraisal, which we share with many others (e.g. Garside et al., 2014), we are quite aware that there is always some sort of appraisal involved when we read and analyse qualitative studies, although it is very challenging to come up with operational criteria that work for different qualitative study types. Still, we have decided to use the CASP-Tool to critically appraise the primary studies included in the review. Please see "Critical appraisal of study quality" in the method's section.	8
5. Application of GRADE CERQual may provide another opportunity for the authors to further articulate the differences in the confidence in review findings from the respective application of QES methods.	Thank you for this hint to the GRADE-CerQual – Tool. We thought about using it, but decided to rely on the longer established ENTREQ tool. "… the 'Enhancing Transparency in the Reporting of Syntheses of Qualitative Research' tool (…) was the first reporting tool for QES and consists of 21 items within the five overarching categories. As a generic tool it documents the most frequently used methods for QES to which it might apply, acknowledging that the approaches	

	and methodology for synthesis are usually driven by the research question posed” (Flemming et al. 2019, p 8) Moreover our interest laid in using a a widely accepted guidance for a protocol be- cause our experience in publishing qualitative research showed so far, that publish- ers (and reviewers) very often refer to the “classical” quantitative paradigm.Tthere- fore we decided to combine PRISMA-P, a checklist, that “is the most widely applica- ble across ***different*** research areas” (Siddaway et al. 2019, p 9.14) with ENTREQ. We added information on use of EPOC for the final review on page 5.	5

Commentary Reviewer 1	Answer	Manuscript Page
6. The author team have not cited the eMERGe guidance on reporting meta-eth- nography or the latest methodological arti- cles on articulating and operationalising the complex analytical phases of meta-eth- nography.	The eMERGe guidance was planned to be used as guidance for the description of the analytical steps of the meta-ethnography in the main paper.	5
7. Of particular note, both the Thomas and Harden method of thematic synthesis and meta-ethnography have commonly been poorly .. applied and not conducted as in-	We very much agree with the point and will try to make appropriate references to the original works. Still, the essence of qualitative methods is what in Ger- man we call “Gegenstandsangemessenheit” (appropriateness with regard to the object of study). We are always called upon to reflect on the appropriate-	

tended. It is common to see published re- views that 'use' these methods but various aspects have been adapted, skipped over or misunderstood. To produce a convincing methodological exemplar, the authors will need to stay faithful to the methods as orig- inally intended and follow the guidance of the originators or authors whose methodo- logical work to further articulate the meth- ods is considered high quality by the origi- nators. Thank you once again for asking me to review this interesting protocol. Overall, comprehensive revision and additional thinking is required.	ness of the methods and be transparent in our methods or methodological choices and to change them when necessary. Rigour should not be confused with methodological orthodoxy. This characteristic of qualitative research re- lates both to orginal research and QES. Meta-Ethnography is a very good exam- ple to show that the origin is not designed as a step by step practice framework (even the authors mention this). Many others (Atkins, Britten, Cahill, Campbell, France, Toye …) refined the methodological steps of meta-ethnography but it is definitely compatible with the epistemology of qualitative research, that the "only and right one" does ***not*** exist. Of more importance in qualitative research is transparency and detailed description of the application of the respective method.	

References:

Barbour, R. S. (2001). Checklists for improving rigour in qualitative research: A case of the tail wagging the dog? *BMJ (Clinical Research Ed.)*, *322*(7294), 1115–1117. https://doi.org/10.1136/bmj.322.7294.1115

Booth, A., Noyes, J., Flemming, K., Gerhardus, A., Wahlster, P., van der Wilt, G. J., . . . Rehfuess, E. (2018). Structured methodology review identified seven (RETREAT) criteria for selecting qualitative evidence synthesis approaches. *Journal of Clinical Epidemiology*, *99*, 41–52.

Flemming, K., Booth, A., Garside, R., Tunçalp, Ö. Noyes, J. (2019): Qualitative evidence synthesis for complex interventions and guideline development. Clarification of the purpose, designs and rele-vant methods. *BMJ Global Health* 4 (Suppl 1), e000882.

Flick, U. (1987). Methodenangemessene Gütekriterien in der qualitativ-interpretativen Forschung. In J. Bergold & U. Flick (Eds.), *Ein-Sichten: Zugänge zur Sicht des Subjekts mittels qualitativer Forschung* (pp. 247–262). Tübingen: dgvt.

Flick, U. (2012). *Qualitative Sozialforschung* (5th ed.). Reinbek: Rowohlt.

Garside, R. (2014): Should we appraise the quality of qualitative research reports for systematic reviews,

and if so, how? *Innovation: The European Journal of Social Science Research* 27 (1), S. 67–79.

Glenton C, Bohren MA, Downe S, Paulsen EJ, Lewin S, on behalf of Effective Practice and Organisation of Care (2019): EPOC Qualitative Evidence Synthesis: Protocol and review template. EPOC Resources for review authors. Oslo, Norway.

Harris, J. L., Booth, A., Cargo, M., Hannes, K., Harden, A., Flemming, K., Garside, R., Pantoja, T., Thomas, J. Noyes, J. (2018): Cochrane Qualitative and Implementation Methods Group guidance series-pa-per 2. Methods for question formulation, searching, and protocol development for qualitative evi-dence synthesis. *Journal of clinical epidemiology* 97, S. 39–48.

Hilari, K., Needle, J. J. Harrison, K. L. (2012): What are the important factors in health-related quality of life for people with aphasia? A systematic review. *Archives of physical medicine and rehabilitation*

93 (1 Supp), S86-S95.

https://doi.org/10.1016/j.apmr.2011.05.028. Lamnek, S. (2010).

Qualitative Sozialforschung (5th ed.). Weinheim / Basel: Beltz.

Lewin, S., Glenton, C., Munthe-Kaas, H., Carlsen, B., Colvin, C. J., Gülmezoglu, M., Noyes, J., Booth, A., Garside, R. Rashidian, A. (2015): Using qualitative evidence in decision making for health and social interventions. An approach to assess confidence in findings from qualitative evidence syntheses (GRADE-CERQual). PLoS medicine 12 (10), e1001895.

Manning, M., MacFarlane, A., Hickey, A. Franklin, S. (2019): Perspectives of people with aphasia post-stroke towards personal recovery and living successfully. A systematic review and thematic synthe-sis. *PLoS ONE* 14 (3). https://doi.org/10.1371/journal.pone.0214200.

Noyes, J., Booth, A., Garside, R. Rashidian, A. (2015): Using qualitative evidence in decision making for health and social interventions. An approach to assess confidence in findings from qualitative evi-dence syntheses (GRADE-CERQual). PLoS medicine 12 (10), e1001895.

Pindus, D. M., Mullis, R., Lim, L., Wellwood, I., Rundell, A. V., Abd Aziz, N. A. Mant, J. (2018): *Stroke survi-vors' and informal caregivers' experiences of primary care and community healthcare services - A systematic review and meta-ethnography.* United States.

Siddaway, A. P., Wood, A. M. Hedges, L. V. (2019): How to Do a Systematic Review. A Best Practice Guide for Conducting and Reporting Narrative Reviews, Meta-Analyses, and Meta-Syntheses. *Annual Re-view of Psychology* 70 (1), S. 747–770.

Stamer, M., Güthlin, C., Holmberg, C., Karbach, U., Patzelt, C., & Meyer, T. (2015). Qualitative Studien in der Versorgungsforschung - Diskussionspapier, Teil 3: Qualität qualitativer Studien [Qualitative Research in Health Services Research - Discussion Paper, Part 3: Quality of Qualitative Research]. *Gesundheitswesen*, *77*(12), 966–975. https://doi.org/10.1055/s-0035-1565235

Strübing, J., Hirschauer, S., Ayaß, R., Krähnke, U. Scheffer, T. (2018): Gütekriterien qualitativer Sozialfor-schung. Ein Diskussionsanstoß. *Zeitschrift für Soziologie* 47, S. 83–100.

Toye F, Seers K, Allcock N, et al. (2013) 'Trying to pin down jelly' - exploring intui-tive processes in quality assessment for meta-ethnography. BMC Medical Research Methodology;13:46.

Reviewer: 2

Reviewer Name

Assoc Prof Mandy Stanley

Institution and Country

Edith Cowan University, Australia

Please state any competing interests or state ‘None declared’:

None declared.

Please leave your comments for the authors below

Commentary Reviewer 2	Answer	Manuscript Page

Thank you for the opportunity to review the manuscript. I am intrigued by the ideas of analysing the data using two different approaches to synthesis and look forward to reading the outcome. It is indeed an interesting question. However by naming it a "double meta-synthesis" seems to me to be misleading. Double synthesis implies to me that you are analysing the data twice over, and whilst essentially you are, it is more that you are approaching the same data in two different ways. It may be a matter of semantics but I think it is important to reflect on	Dear Prof. Stanley, first of all we would like to thank you for your helpful comments on our manuscript. The title has been changed to “Healthcare related needs and desires of older people with post-stroke aphasia: study protocol of a systematic review and qualitative evidence synthesis using two different approaches”.	1
You say you will search at least four databases. Will it be four or more than four?	Please see new description under “databases and search terms” in the method’s section.	6
There appears to be some material out of place in the ethics and dissemination section of the abstract.	Thank you for this hint, we revised the text.	
	The point of departure for our research question comes from the problems we see in the German health care system with regard to cross-sectoral care. While the different health care institutions caring for stroke patients all have their merits, there is a huge problem in coordination of care and taking responsility of care, which is often left to pa-	

Will you inform policy? How? Much more likely to in- form best practice.	tients overwhelmed with their situation. Especially patient with aphasia have a literal problem to raise their voice. Therefore, we want to learn from qualitative studies on the needs and desires (i.e. objective needs as reported by the patients themselves, as well as subjective needs re- lated to goals, preferences, expectations etc.) in order to inform pre- sent health care approaches that aim to integrate the different care stakeholders, especially rehabilitation, nursing and primary helath care services.	2
I was interested in the decision to focus on older peo- ple (rather than elderly) yet you cite statistics about the frequency of stroke from age 40-79.	We added some new research more focused on the age group 65 years and older (see Introduction).	2
The manuscript needs review for a number of missing words and some smoothing out of English language.	The manuscript has been revised.	
I got to the end of the introduction section and was unclear how the findings about needs related to peo- ple with aphasia, and why the review needed to be conducted. This needs to be much more explicit, not just the methodological curiosity in trying two differ- ent analysis approaches.	Please see the revised introduction.	3

In the methods section there is a statement about eth- ics which is not necessary here as stated later.	We placed the ethics statement at the end of the paper.	11

Commentary Reviewer 2	Answer	Manuscript Page
Databases and search terms: state the lan- guages to be searched in	We did not restrict the search to certain languages.	
Table 2 objective Is it consented or consen- sus?	Text changed to consensus-based.	Table 2
Data extraction - will you also extract study design?	We added “Study design” (see “Data extraction and analysis”)	9
You talk about the distinction between first and second order constructs not being clear, what approach will you use?	Because it may confuse readers, we decided to remove the sentence about this problem.	
The section on how you chose the two dif- ferent approaches needs to come earlier to make a bit more sense.	This part was modified and shifted to the introduction.	3
Am I correct in understanding that the analysis will be done by two independent teams? I was wondering how you would	Discussing findings with other researchers, so called triangulation, is a common procedure in qualitative research and aims to prevent scientists from overlooking further interpretations. One may call this influencing each other, but in qualitative	

prevent the findings from one influencing the other especially if they are both dis- cussed with the research network.	research this is deemed to be adequate because it may enhance the validity of the findings.	
The discussion is a bit weak. Aphasic pa- tients should be people living with aphasia. You suggest it will inform more patient- centred ways but how do you know cur- rent ways are not? The findings should provide a more in-depth conceptual under- standing of what is important for people living with aphasia..	We revised the term aphasic people to people living eith aphasia. Studies supporting our view that current ways are not adequately patient-cen- tered (see first paragraph on p.3): “Several quantitative investigations explored whether stroke survivor’s needs are addressed adequately in the health care system [5–11]. All of them revealed a substantial proportion of individuals with unmet needs from at least 51% [9] up to 100 % [8, 12]. These studies clearly show that existing unmet needs are related to the severity of disease or disability as well as psychosocial aspects, living in de- prived areas, low income or being member of an ethnic minority group.”	3
I am also intrigued about how you will manage reporting. If you publish one ap- proach will that make publication of the other approach more difficult? It clearly will lead to an interesting methodological	We plan to publish both reviews and an additional one about the methodological challenges and how we solved them.	

paper however that shouldn't be the pri- mary goal		

References:

Busch MA, Kuhnert R. 12-Monats-Prävalenz von Schlaganfall oder chronischen Beschwerden infolge ei-nes Schlaganfalls in Deutschland. *Journal of Health Monitoring* 2017;2(1):70–76

Kohler M, Deutschbein J, Peschke D, et al. Stroke occurence in Germany - on the comparability of insur-ance data and registry-based data. *Fortschr Neurol Psychiatr* 2014;82(11):627–33

Stahmeyer, J. T., Stubenrauch, S., Geyer, S., Weissenborn, K. Eberhard, S. (2019): Häufigkeit und Zeit-punkt von Rezidiven nach inzidentem Schlaganfall. *Dtsch Arztebl International* 116 (42), S. 711– 717.

Reviewer: 3

Reviewer Name

Betje Schwarz

Institution and Country

German Pension Insurance Berlin-Brandenburg,

Germany

Please state any competing interests or state ‘None declared’:

None declared.

Please leave your comments for the authors below

Kommentar Reviewerin 3	1. Antwortvorschlag / Anmerkung	Ggf. Quellenangabe

	2. Formulierungsvorschlag neuer Text	
A very good study protocol of a very in- teresting and promising study	Dear Mrs. Schwarz, thank for your praise of our manuscript and the efforts spend on reviewing it.	
I have only one request for clarification: in the section 'Authors Contributions' you state, that "All authors contributed to […] the risk of bias assessment." Which bias assessment?	Thank you for this hint. The sentence related to risk assessment was printed unintentionally and now has been deleted.	
You also state, that you will not assess the methodological quality of the pri- mary studies: "Even though Thomas & Harden [28] describe quality appraisal as a step in their approach to thematic synthesis it has no consequences for study inclusion or exclusion or the weighing of results. Following Noblit & Hare [29] meta-ethnography does not necessarily include quality appraisal of the primary studies. Therefore, we de-	We revised the part on critical appraisal.	8

cided not to include explicit quality as- sessment of the included studies."			

VERSION 2 – REVIEW

REVIEWER	Professor jane Noyes Bangor University, UK.
REVIEW RETURNED	09-Nov-2020

GENERAL COMMENTS	Thank you for resubmitting your review. The amendments are acknowledged. The authors have acknowledged and addressed some feedback and argue strongly against other feedback. This is a shame as the unresolved feedback could have further strengthened the protocol and further refined and clarified the scope of the work. Fundamentally, this is a protocol and the authors will subsequently find out for themselves the known challenges/similarities/differences of the methods that they are proposing and to a lesser extent comparing. Most people accept that the initial stages of Thomas and Harden's approach to thematic synthesis and Noblet and Hare's meta-ethnography are very similar. The only difference is that with meta-ethnography you can potentially go further if you have the right type of qualitative evidence to develop new theory. It remains to be seen if the authors will have access to the type of qualitative evidence that enables theory development. If not they will likely produce two reviews with the same findings. It remains challenging to see the merit in wanting to use two synthesis methods with more similarities than differences and the author's case is unconvincing. Nonetheless, if they want to spend time doing it and to gain some experience of applying two QES synthesis methods, then why not. The team will likely gain valuable experience and further their QES skills. There are several remaining misconceptions and confusions in the manuscript. The authors misunderstand the meta-ethnographic processes of translation, reciprocation and reinterpretation. Both first and second order constructs are needed to do this. The authors should not rely on the primary study author's findings as the purpose is to come up with new interpretations not seen in the primary studies. The authors are confused about the purposes of ENTREQ and GRADE CERQual. One is a reporting guideline (ENTREQ), the other is an appropach for assessing the confidence in synthesised findings. Overall, unless the authors can provide a convincing evaluation as to the strengths and weaknesses of two QES synthesis methods that are more similar than different, it is difficult to justify the

	inclusion of two synthesis methods in a single protocol. If they however want to gain the experience as a team then why not. This could be a valuable learning exercise to increase the team's capabilities. Qualitative studies and qualitative evidence syntheses produce findings and not results. The manuscript still refers to 'results' in a few places. Qualitaty appraisal of primary qualitative studies is now included in the protocol. Nontheless, the text refers to quality appraisal scores. Calculating scores is not considered appropriate as not all domains of quality are considered equal and therefore total quality scores are considered meaningless. What is considered more important is the identification of methodological limitations in primary qualitative studies studies and how these methodological limitations impact on the interpretation of findings. It is not clear why book chapters are included? It is very unlikely that qualitative studies will be published in sufficient detail to appraise and extract in book chapters. Rich qualitative data may however be found in theses and grey literature reports of qualitative studies (for example, those QES studies undertaken by third sector organisations). The latter sources of qualitative evidence are not mentioned.

REVIEWER	Assoc Prof Mandy Stanley Edith Cowan University, Australia
REVIEW RETURNED	11-Nov-2020

GENERAL COMMENTS	Thank you for comprehensively addressing my comments on the first submission. You have addressed all the points that I have raised. My further comments relate mostly to editing of language to increase clarity. Title: this is a much better reflection of what you plan to do. Abstract: I know use of the word elderly is common but older people is preferable. People live with aphasia post stroke rather than suffer. You use the abbreviation QES in the abstract but it needs to be in full. The sentence about the QES at the end of the introduction appears to come out of nowhere. In the methods section I think you aim to synthesis rather than summarise. Strengths and limitations of the study dot points: Could one of the strengths be about the content rather than the researchers. I think one of the strengths is taking the two approaches to synthesis. Introduction 1st paragraph "While we know about the merits of acute care for stroke.." Who is we in this sentence? I think you are using the collective we. Flip the sentence around to while the merits of acute care for stroke.... PICoS; you could refer the reader to table 1 at this point although in the table the second P is labelled differently. Table 2. I had great difficulty in understanding this process and why all 4 processes are listed. Maybe it is just me. Author background and reflexivity: I think this section could be written more simply and clearly. Can you summarise to say that the authors come from a range of backgrounds including x and x, and have a depth and breadth of experience in qualitative analysis? I was interested that no-one comes from speech pathology. Discussion: in the last paragraph I think you mean adapt the questions rather than adopt.

VERSION 2 – AUTHOR RESPONSE

Reviewer 1	
Thank you for resubmitting your review. The amendments are acknowledged. The authors have acknowledged and addressed some feedback and argue strongly against other feedback. This is a shame as the unresolved feedback could have further strengthened the protocol and further refined and clarified the scope of the work.	Dear Mrs. Noyes, Thank you for your precious time spent on studying our manuscript a second time and your helpful comments. Please find our reply to your comments below.
The authors misunderstand the meta-ethnographic processes of translation, reciprocation and reinterpretation. Both first and second order constructs are needed to do this. The authors should not rely on the primary study author's findings as the purpose is to come up with new interpretations not seen in the primary studies.	Perhaps we are talking about different interpretations of the same. Our understanding of metaphor or concept (the terms usually used in the description of the process of translation) is, that – even though it may literally be what a study participant said – it is a second order construct because it is an attribution coming from the researcher, not from the participant. Even if identical with a quote, if classified as metaphor, key-metaphor, concept etc. it was selected out of other things the interviewed person said. This selection is already an interpretative step made by the study authors and therefore a second order construct. In France et al. 2019 you and your colleagues stated, that "first order constructs **can** be analysed and synthesised along with their corresponding second order constructs but not in isolation." There is no obligation to do so mentioned. In our paper we stated that we are going to include first order constructs (= quotations of the participants, described as "interpretations of the participants" by Schütz) if clarification of the second order construct is necessary. We do not see any violation of the concept of meta-ethnography. To support our point, we present below some of our evidence for using second order constructs.

	However, **accessing first order constructs, or participant views or beliefs, is problematic** in the context of a meta-ethnography **since the data extracts included in the primary papers have already been selected** from the full dataset by the study authors. These extracts therefore do not reflect the totality of participant experiences**. Author, or second order, interpretations, can provide more insight** by offering an explanation of the observed phenomena, but we found that the level of interpretation offered in the papers was minimal – many of the papers were highly descriptive in nature **Step 5: Translating studies into one another** The original method implies comparing the metaphors and concepts in one account with the metaphors and concepts in others (…) In some examples, third order interpretations seemed to be derived from first and second order constructs reported in the primary studies [4,14,33], but in most cases the process for doing this was not clearly defined. (…) As the process of synthesising research in meta-ethnography is not clearly delineated, we agreed on a method of synthesis based on our reading of a number of existing reviews. (…) **we listed the translated themes and subthemes in a table, juxtaposed with secondary themes derived from author interpretations.**	Cited from: Atkins, S., Lewin, S., Smith, H., Engel, M., Fretheim, A., & Volmink, J. (2008). Conducting a meta-ethnography of qualitative literature: Lessons learnt. BMC Medical Research Methodology, 8(1), 21. https://doi.org/10.1186/1471-2288-8-21 P 6-7

Reviewer 1		
	Reading the studies This stage involved the careful reading of the chosen papers in order to identify the **main concepts.** Determining how the studies are related We looked across the different papers for common and recurring	Cited from: Britten, N., Campbell, R., Pope, C., Donovan, J., Morgan, M., & Pill, R. (2002). Using meta ethnography to synthesise qualitative research. A worked example. Journal of Health Services Research & Policy, 7(4), 209–215. https://doi.org/10.1258/135581902320432732 (p 211)

	concepts. These **key concepts** were: ... In order to be explicit about how the concepts compared with one another, we created a grid into which we placed the concepts from each paper (see row labels in Table 1). The first four rows of the grid include relevant details of the study setting and research design. These methodological details are essential contextual information for the synthesis. From the fifth row onwards, each row of the grid represents a **key concept**. In developing the grid, we used Schutz's notion of first- and second-order constructs. Schutz used the term Ž first order construct to refer to the everyday understandings of ordinary people and the term second-order construct to refer to the constructs of the social sciences. Thus, in Schutz's terms, **the explanations and theories in the last row of the grid are <u>second-order interpretations</u>.**	
	"Phase three: Reading the studies Noblit and Hare (1988) advised repeated reading of the studies and noting or extracting the key concepts or metaphors that existed in each study. (…) **These <u>key concepts</u> or <u>metaphors</u> <u>are the 'data'</u> for the synthesis**. (p 5) Phase five: Translating the studies into one another **The translation process entails <u>examining</u> the <u>key concepts</u>** within and across the studies and is similar to the method of constant comparison. The translations can be conducted in chronological order (Campbell et al., 2011). Each concept from each paper is compared with all the other papers in turn to check	Cited from: Cahill, M., Robinson, K., Pettigrew, J., Galvin, R., & Stanley, M. (2018). Qualitative synthesis: A guide to conducting a meta-ethnography. British Journal of Occupational Therapy, 81(3), 129–137. https://doi.org/10.1177/0308022617745016

	for the absence or presence of concepts." (p 6)	
	"Meta-ethnography involves selecting relevant empirical studies to be synthesised, reading them repeatedly and **noting down key concepts (interpretive metaphors**). **<u>These key concepts become the raw data for the synthesis</u>**. Noblit and Hare suggest that synthesis is achieved through reciprocal translation; this entails examining the key concepts in relation to others in the original study, and across studies, and is analogous to the method of constant comparison used in qualitative data analysis. The translation of **key concepts or interpretive metaphors** …" (p 673) As we indicated in the introduction, the **purpose of a synthesis of qualitative research** is to achieve a level of conceptual development beyond that attained in the individual studies included: in effect **<u>to move from second-order</u> to third-order** constructs. (p 681) We found the appraisal process a useful prelude to a synthesis because it helped to screen out inappropriate papers and one of poor quality. It also provided the opportunity for i**dentifying the <u>concepts or interpretive metaphors, which formed the raw material</u> for the synthesis."** (p 682)	Cited from: Campbell, R., Pound, P., Pope, C., Britten, N., Pill, R., Morgan, M., & Donovan, J. (2003). Evaluating meta-ethnography. A synthesis of qualitative research on lay experiences of diabetes and diabetes care. Social Science and Medicine, 65, 671–684.

Reviewer 1	

	France et al. criticize: “A number of papers (e.g., S5, S8, S9, S10, S11, S13, S17, S25, S32) **drew on both participant quotes and primary study authors’ concepts rather than just the latter in the translation process – this differs from how Noblit and Hare described translation** but seems to be fairly common practice [44]. Most reviewers stated that they had conducted a synthesis of translations but not how, for instance, a number of them (papers S1, S5, S20, S24, S29) said **they had developed third order constructs from second order constructs**, but not how they had done this.”	Cited from: France, E. F., Ring, N., Thomas, R., Noyes, J., Maxwell, M., & Jepson, R. (2014). A methodological systematic review of what’s wrong with meta-ethnography reporting. BMC Medical Research Methodology, 14(1), 119. https://doi.org/10.1186/1471-2288-14-119 p. 11
	“Phase 4 Determining how the studies are related” Noblit and Hare [9] described the process of Phase 4 as :‘the various studies must be “put together.” This requires determining the relationships between the studies to be synthesized. We think **it makes sense** to create a list of the **key metaphors, phrases, ideas, and/ or concept**s (and their relations) used in each account and to juxtapose them.’ ([9], p. 28)”	Cited from: France, E. F., Uny, I., Ring, N., Turley, R. L., Maxwell, M., Duncan, E. A. S., . . . Noyes, J. (2019). A methodological systematic review of meta-ethnography conduct to articulate the complex analytical phases. BMC Medical Research Methodology, 19(1), 35. https://doi.org/10.1186/s12874-019-0670-7 page 2
	„A meta-ethnography seeks to go beyond single accounts to reveal the analogies between the accounts. It reduces the accounts while preserving the sense of the account through **the selection of key metaphors and organizers**. The “senses” of different accounts are then translated into another.” (…) “… **when we talk about the key metaphors of a study, we are referring to what others may call the themes, and / or concepts revealed by qualitative studies**.	Cited from: Noblit, G. W., & Hare, R. D. (1988). Meta-ethnography: Synthesizing qualitative studies (1. printing). Qualitative research methods: Vol. 11. Newbury Park Calif. u.a.: Sage Publ. p. 13-14”
	“We then translated **the findings** from the studies into each other within each of the medicine groups. To clarify how the findings related to each other, we developed ‘**maps’ of the key**	Cited from: Pound, P., Britten, N., Morgan, M., Yardley, L., Pope, C., Daker-White, G., & Campbell, R. (2005). Resisting medicines: a synthesis of qualitative studies of medicine

	findings for each of the groups by drawing the relationships between them.	taking. Social Science and Medicine, 61, 133–155. p. 135
	The 'data' of Meta-ethnography are second order constructs. In meta-ethnography, these second order constructs are then further abstracted to develop third order constructs (the researchers' interpretations of the original authors' interpretations). The second order interpretation exemplified by narrative is based on a body of knowledge accessed through fieldwork. Therefore, attempts to re-organise findings without access to this wider body of knowledge might not illuminate the conceptual interpretation originally intended	Cited from: Toye, F., Seers, K., Allcock, N., Briggs, M., Carr, E., & Barker, K. (2014). Meta-ethnography 25 years on: Challenges and insights for synthesising a large number of qualitative studies. BMC Medical Research Methodology, 14, 80. https://doi.org/10.1186/1471-2288-14-80 p. 7

Reviewer 1	
The authors are confused about the purposes of ENTREQ and GRADE CERQual. One is a reporting guideline (ENTREQ), the other is an appropach for assessing the confidence in synthesised findings.	We fully agree that ENTREQ is a reporting guideline. In our manuscript we describe that we follow the ENTREQ checklist in writing the study protocol and reporting the findings from the review. This is in line with its intended use. Indeed there was a misunderstanding about your comment/advise on the use of GRADE-CERQual in your last review. We consider the use of GRADE-CERQual as helpful and added this to our manuscript (page 5: "Moreover we will use the GRADE-CERQual-series to guide us through the procedure of assessing confidence in the evidence from the two reviews [36–40]."
Overall, unless the authors can provide a convincing evaluation as to the strengths and weaknesses of two QES synthesis methods that are more similar than different, it is difficult to justify the inclusion of two synthesis methods in a single protocol. If they however want to gain the experience as a team then why not. This could be a valuable learning exercise to increase the team's capabilities.	As we already pointed out in the first review-process, our interest is to gain insights in the practical challenges and their possible implications. (p. 11): "We have decided to choose approaches similar enough to be able to apply to the same research question (leaving it open to adapt the question during the course of the study if necessary), and to base (!) it upon the same set of studies, which does not predetermine the final selection of studies or the priorities given to each study. We assume, however, that the reality of choice over QES approaches is in practice much more dependent on criteria that go beyond methodological considerations – such as personal background and familiarity with a method, personal preferences of team members etc. In practice, therefore, boundaries between different QES approaches might be quite blurred in terms of concepts, methods or terms. Since there is no "ideal" comparison in this vein, we find it appropriate to make a few restrictions to the respective QES methodologies in order to allow for a substantive comparison. It is well acknowledged that these

	restrictions should be an explicit part of the approach to comparison."
Qualitative studies and qualitative evidence syntheses produce findings and not results. The manuscript still refers to 'results' in a few places.	We edited the manuscript in accordance to your comment. Thanks for pointing that out to us. Either way, we would like to share some thoughts on this point. A strict distinction between "results" and "findings" is not generalizable. Even though the term "findings" may be used more frequently in qualitative research, there is no obligation to avoid the term "results". Perhaps we do not see the brisance of the term "results" because in german language we only have one term (Ergebnis) to describe results or findings. But also we have come across native speakers who use the term "results": "Transferability refers to the degree to which the results of qualitative research can be generalized or transferred to other contexts or settings" (Qualitative research in applied linguistics, Heigham & Croker 2009).
Qualitaty appraisal of primary qualitative studies is now included in the protocol. Nontheless, the text refers to quality appraisal scores. Calculating scores is not considered appropriate as not all domains of quality are considered equal and therefore total quality scores are considered meaningless. What is considered more important is the identification of methodological limitations in primary qualitative studies studies and how these methodological limitations impact on the interpretation of findings.	We cited Pindus et al.'s view on quality scores, but as well other authors not referring to scores but to quality appraisal in general. Moreover we stated that we are going to use the 'Critical Appraisal Skills Programme (CASP) Checklist for qualitative studies', which does not include any kind of score calculation.
It is not clear why book chapters are included? It is very unlikely that qualitative studies will be published in sufficient detail to appraise and extract in book chapters.	We do not understand this point. A book chapter may be much more comprehensive than a journal article. If your critique would go ahead to the lacking peer review process in book publications, one might think about it. But keeping book chapters aside just because of possible shortage of description might be in conflict with the argument relating to the length of the manuscript.
Rich qualitative data may however be found in theses and grey literature reports of qualitative studies (for example, those QES studies undertaken by third sector organisations). The latter sources of qualitative evidence are not mentioned.	We searched a database for grey literature, please see manuscript page 6 "The refined MEDLINE strategy was adapted to PsycINFO, Psyindex, CINAHL, Web of Science (Social Science Citation Index), ASSIA and a database indexing grey literature: BASE."

Reviewer: 2	

Comments to the Author	
Thank you for comprehensively addressing my comments on the first submission. You have addressed all the points that I have raised. My further comments relate mostly to editing of language to increase clarity.	Dear Prof. Stanley, thanks for spending your precious time in another round of reviewing our manuscript. We appreciate your helpful comments.
Title: this is a much better reflection of what you plan to do.	Thank you for that positive comment.
Abstract: I know use of the word elderly is common but older people is preferable. People live with aphasia post stroke rather than suffer. You use the abbreviation QES in the abstract but it needs to be in full. The sentence about the QES at the end of the introduction appears to come out of nowhere. In the methods section I think you aim to synthesis rather than summarise.	1. We changed elderly to older and suffering from to living with. 2. We changed QES to qualitative evidence synthesis/syntheses. 3. The second last sentence introduces the idea of two qualitative evidence syntheses which is picked up again in the following sentence. Unfortunately we cannot comprehend your feeling that the last sentence seems to come out of nowhere. Either way we edited the last two (now three) sentences to increase comprehensibility. 4. You are right, we changed summarize to synthesize.
Strengths and limitations of the study dot points: Could one of the strengths be about the content rather than the researchers. I think one of the strengths is taking the two approaches to synthesis.	Sure, we are happy to add another dot point corresponding to your comment.
Introduction 1st paragraph "While we know about the merits of acute care for stroke.." Who is we in this sentence? I think you are using the collective we. Flip the sentence around to while the merits of acute care for stroke....	We have edited the sentence accordingly.
PICoS; you could refer the reader to table 1 at this point although in the table the second P is labelled differently.	We added a reference to table 1.
Table 2. I had great difficulty in understanding this process and why all 4 processes are listed. Maybe it is just me.	Table 2 explicates the four steps (identification, screening, eligibility assessment and inclusion) of the study selection process described in: Moher D, Liberati A, Tetzlaff J, et al. Preferred reporting items for systematic reviews and meta-analyses: The PRISMA statement. BMJ 2009;339:b2535. The table was included to make our study selection process comprehensible and transparent. We edited the table to make it easier to comprehend.
Author background and reflexivity: I think this section could be written more simply and clearly. Can you summarise to say that the authors	Author reflexivity and transparency about professional backgrounds is essential for judging on findings from qualitative research. The extensive description was asked for

come from a range of backgrounds including x and x, and have a depth and breadth of experience in qualitative analysis? I was interested that no-one comes from speech pathology.	by the other reviewer. Therefore, we decided to keep the detailed passage.
Discussion: in the last paragraph I think you mean adapt the questions rather than adopt.	We corrected that.

VERSION 3 – REVIEW

REVIEWER	Jane Noyes Bangor University UK
REVIEW RETURNED	15-Jan-2021

GENERAL COMMENTS	No additional comments

REVIEWER	Associate Professor Mandy Stanley Edith Cowan University
REVIEW RETURNED	22-Jan-2021

GENERAL COMMENTS	Thank you for addressing my previous comments. Having come back to reading this manuscript again I still think that there is too much emphasis on the academic exercise in conducting the evidence synthesis using too different approaches rather than on the topic of health needs of people living with aphasia. Granted that is an interesting academic exercise and one that as a qualitative researcher and someone who does meta-ethnography has great interest in however that is for a methodology paper where the title of this paper frontloads the topic. I still think the description of the researchers' positions is over complicated and could be stated more simply and clearly however I can see why the authors have retained that section. I leave that to the Editor to decide. The writing and language use could do with a bit tighter editing. The first paragraph of the introduction is very long and could be made into two. The two paragraphs about the literature and establishing the need for the review could be re-written to make the points stronger. Eg. Quantitative studies of stroke survivor's needs have revealed unmet needs ranging from x to x (references).

VERSION 3 – AUTHOR RESPONSE

Reviewer: 1

Dr. Jane Noyes, Bangor University

Comments to the Author:

No additional comments

Answer: *Thank you.*

Reviewer: 2

Prof. Mandy Stanley, Edith Cowan University

Comments to the Author:

Thank you for addressing my previous comments.

Having come back to reading this manuscript again I still think that there is too much emphasis on the academic exercise in conducting the evidence synthesis using too different approaches rather than on the topic of health needs of people living with aphasia. Granted that is an interesting academic exercise and one that as a qualitative researcher and someone who does meta-ethnography has great interest in however that is for a methodology paper where the title of this paper frontloads the topic.

Answer: *Dear Prof. Stanley, thank you for taking the time to review our manuscript once again. We understand that some readers might be more interested in the topic of our review and other readers might be more interested in the methodological discussion. The title was set up to suit both groups. However, we also see the tension between addressing the topic and the methodological exercise. To avoid a frontloading of the topic, which might not represent the content of the paper appropriately, we decided to rearrange the title without changing the content: Study protocol of a systematic review and qualitative evidence synthesis using two different approaches: Healthcare related needs and desires of older people with post-stroke aphasia.*

I still think the description of the researchers' positions is over complicated and could be stated more simply and clearly however I can see why the authors have retained that section. I leave that to the Editor to decide.

Answer: *We admit, that this section might seem very detailed and long. As the description and reflection of professional backgrounds is information asked for by several checklists on reporting quality for qualitative research [e.g. COREQ by Tong et al. (2007) and the EPOC Protocol and Review Template by Glenton et al.*

(2019)] we would prefer to keep the section in the form and extent presented in the manuscript. This was also asked for by Reviewer No.1 and the editor did not object. Either way, we made some small changes to make this section more comprehensible.

The writing and language use could do with a bit tighter editing. The first paragraph of the introduction is very long and could be made into two. The two paragraphs about the literature and establishing the need for the review could be re-written to make the points stronger. Eg. Quantitative studies of stroke survivor's needs have revealed unmet needs ranging from x to x (references).

Answer: *We divided the first paragraph into several paragraphs and adapted the text according to the proposed changes.*